

COURT FILE NO.: 04-B7133
DATE: 2007-01-23

ONTARIO
SUPERIOR COURT OF JUSTICE

B E T W E E N:

Derek Gordon, Dianne Gordon, and William
Gordon

Plaintiffs

- and -

Cory Greig, Ford Credit Canada Leasing
Company

Defendants

Ryan Morrison, Karen Morrison, Peter
Morrison and Rochelle Morrison, by her
Litigation Guardian, Karen Morrison

Plaintiffs

Corey Greig, Ford Credit Canada Leasing
Company

Defendants

) R. Oatley & W. Teggart, for the Plaintiffs

) C. Godden & M. Healy, for Cory Greig
) and M. Shulgan & A. Stein, for Ford Credit
) Canada Leasing Company

) Court File No.: 04-B7066

) J. Vigmond & B. Cameron, for the Plaintiffs

) C. Godden & M. Healy, for Cory Greig and
) M. Shulgan & A. Stein, for Ford Credit
) Canada Leasing Company

) **HEARD:** Oct 10-13, 16-20, 23-27, 30-Nov
3, Nov 15-17, 20-24, 27-Dec 1, Dec 6-8, 11,
12/ 2006

Justice B.A. Glass

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Introduction

[1] These reasons cover two actions flowing from the same motor vehicle accident in which Cory Greig drove a motor vehicle owned by Ford Credit Canada Leasing Company. Ryan Morrison and Derek Gordon were passengers in the Greig vehicle. Since the commencement of proceedings, the Defendant Gore Mutual has been released from the action.

[2] On September 26, 2003, Cory Greig drove his motor vehicle from the Bradford, Ontario area to the accident scene on City of Kawartha Lakes Road # 4 west of Little Britain. He was traveling in excess of the speed limit of 80 km per hour and the recommended speed for a very sharp corner. The recommended speed was 50 km per hour. The driver passed two vehicles while proceeding into the curve and pulled back into the driving lane in order to avoid a collision with an approaching vehicle. Mr. Greig pulled too far into the driving lane and struck the shoulder. He lost control of his truck and veered back across the road and rolled the truck several times.

[3] Mr. Greig did not have his seat belt hooked up. Mr. Gordon was the passenger in the right front seat, and he did not activate his seat belt. Mr. Morrison was the passenger in the passenger right rear seat, and he did not connect his seat belt. A fourth person was in the rear left passenger

seat. He had engaged his seat belt. The three persons without seat belts engaged were thrown out of the vehicle.

[4] Mr. Morrison experienced a T5-6 injury to his back and is in a wheelchair. Mr. Gordon received severe brain injuries and neck injuries.

[5] Both men are now 25 years of age. They have claimed damages for pain and suffering, loss of past wages, loss of future wages and the cost of future care. The other plaintiffs have brought family law claims.

[6] There was an issue of contributory negligence when the trial began; however, counsel advised that they have settled the issue of liability. This will result in an automatic adjustment to my award of damages to reflect the percentage of contributory negligence that counsel have agreed should be attributed to each of the injured men. These reasons accordingly assess the damage claims only.

Issues

[7] The quantum of loss of past wages.

[8] The quantum of loss of future income.

[9] The quantum of the cost of attendant and future care.

[10] The appropriate discount rate to be used in the Morrison action.

[11] Any allowance for contingencies.

[12] The quantum of general damages.

[13] The quantum of family law claims.

Background of the Facts for Both Actions

[14] The driver, Cory Greig, was a personal friend of the two Plaintiffs, Ryan Morrison and Derek Gordon. They were average young men who enjoyed partying and consuming alcoholic beverages on weekends. On the night of September 26, 2003, the four men were to travel to the cottage of the parents of Cory Greig in the Lindsay area. Mr. Morrison was to be the designated driver. When they set out, they left in the Morrison vehicle; however, the vehicle was malfunctioning so that they returned to Bradford and took Mr. Greig's vehicle with Mr. Greig driving. They had beer in the truck and consumed some.

[15] Mr. Morrison had a beer in the truck. He did not see the others drinking a lot of beer in the truck.

[16] The blood/ alcohol concentration in the blood of Mr. Greig was determined by Dr. Corbett to be between 142 milligrams and 192 milligrams in 100 millilitres of blood. The limit permitted in Canada is 80 milligrams in 100 millilitres of blood.

[17] Blood/alcohol concentration levels for Ryan Morrison calculated by Dr. Corbett to be less than 21 milligrams at the time of the accident.

[18] Cory Greig's parents had a cottage in the Lindsay area for several years. It appears that he was familiar with the road on which he was driving on September 26th. When presented with a hypothetical fact situation regarding Cory Greig, Dr. Corbett opined that if a person had a historical drinking pattern as did Mr. Greig, he might not exhibit signs of intoxication. This was a question in relation to the passengers getting into the Greig vehicle when his ability to drive might have been impaired by the consumption of alcohol. Ryan Morrison Gordon had not thought that Mr. Greig was impaired when they set off to his parents' cottage. Derek Gordon did not have any memory of the trip.

[19] The injured persons were transported to the Ross Memorial Hospital in Lindsay and subsequently to Sunnybrook Hospital in Toronto. Both men remained in hospital for an extensive time and were later transferred for rehabilitation assistance to Toronto Rehabilitation Institute.

[20] Being released from hospital was the beginning of a lifelong program of coping with the injuries sustained from the accident.

[21] Basically, the need for continuous attendant care for both Plaintiffs as well as continuous rehabilitation assistance for Ryan Morrison will be provided throughout their lives.

[22] The primary care-giving persons are the families of each Plaintiff. Dianne Gordon and Karen Morrison are the mothers of the two Plaintiffs, and they are the main persons assisting the men. There can be no doubt that each family and especially each mother have risen to the challenge of helping an injured family member. The degree of constant, selfless care given by each mother is remarkable.

[23] Liability is not an issue in these trials. Rather, a determination of the quantum of damages is the focus of this trial. There is a major difference of positions between the Plaintiffs and the Defendants in the compensation that the court should consider for future care costs as well as future income considerations. The bottom line is that each of the injured Plaintiffs seeks about \$10 million in total compensation and the Defendants recommend less than half that quantum.

[24] The reasons will be divided into the following categories for each Plaintiff:

- (i) attendant care;
- (ii) housekeeping and home maintenance;
- (iii) the cost of future care;

- (iv) the loss of past income;
- (v) the loss of future income;
- (vi) whether there ought to be any adjustment in the discount rate to be applied with respect to Ryan Morrison;
- (vii) whether there should be a management fee in each action;
- (viii) general damages;
- (ix) any allowance for contingencies;
- (x) family law claims.

The Gordon Action

[25] No one disputes that Derek Gordon experienced severe injuries. They include severe acquired brain injury and upper spinal injury. Bladder and bowel control, sense of smell, sense of taste, sense of hunger, and sense of temperature have been affected by the injuries sustained. This young man was placed in a halo because of upper spine and neck injuries encountered after he was thrown out of the vehicle. Not only did his brain encounter swelling and bruising, but some brain shrinkage occurred as well. Dr. Cooper, an expert witness in the field of neuroradiology, thought that the injuries to the brain were consistent with a person being bounced around so that the head received several blows. Exhibits 48—53 provide diagrams illustrating the fractures to the skull, the cervical spine, initial brain injury findings on CT Scans, and bleeding within the brain. In time, Mr. Gordon was able to walk and talk. Although he functions, he does not enjoy a normal life.

[26] In order to engage in sexual intercourse, he must use prescribed medication for erectile dysfunction. He does not enjoy an interactive relationship with women his own age because of a limited ability to carry on a normal communicative interchange of conversation and existence with others. He does not understand activities in life as most people do. He is a person who thinks in concrete terms and he has poor judgment. Derek has a tendency to express racially inappropriate comments, which makes others feel uncomfortable to be in his company. Although he might commence a relationship with a young woman, in time it becomes apparent to the companion that Derek is not as complete a young man as the other might have thought at first. His lack of bladder and bowel control requires him to use special undergarments. He tries to make a habit of going to the toilet hourly in order to avoid a mishap. It is understandable that this generates a difficulty in maintaining a relationship with another person.

[27] His frustration level is low. He will punch a wall when he becomes angry. He will talk roughly to his mother and others. Mr. Gordon experiences depression. He has difficulty in concentrating. He cannot pay attention for very long, and he has difficulty in concentrating.

[28] Derek's memory is impacted significantly. He simply has considerable difficulty remembering what has been said or what he is to do. For example, when he was being examined in chief by Mr. Oatley, he was providing an answer that did not appear to be focusing on the question. Mr. Oatley asked Derek if he remembered what the question was. The answer was that he did not recall. This might be considered as not much of an example, but this was during the most important event in his life since this motor vehicle accident, i.e. the trial. People might daydream or not pay attention to what others say to them, but not during an event that the individual states is important to him. Derek testified that he hopes that his life will become a good existence after the trial as if such an outcome will be automatic after the trial. He stated that his life is not very nice currently. Therefore, one would anticipate that he would pay close attention to questions being asked and direct answers to those questions. Unfortunately, Derek simply did not remember the question his own lawyer had asked him. I take that example as a clear demonstration of a young man with acquired brain injury who does have a reduced ability to remember.

[29] Dr. Elizabeth Bosman is a neuropsychologist who advised that Mr. Gordon brain injury was severe as confirmed by his low score on the Glasgow Coma Tests. The brain injury was in the category of being catastrophic.

[30] Derek Gordon drives a motor vehicle. His family members have concerns about whether or not he should be driving a motor vehicle, but he has satisfied the licence-issuing agency for Ontario. Robert Brown testified as a driving assessor and advised that he was satisfied that Derek could drive a motor vehicle. He conducted an assessment after driving with Derek on several occasions. Mr. Brown commenced driving with Derek at times when Derek was well rested and at times of the day when he would be less likely to become frustrated. As time passed, the driving was advanced to 400 category highways with more traffic so that Mr. Brown could determine the ability of Derek to cope with more stressful situations. His conclusion was that Derek Gordon could drive on such highways. However, Mr. Brown noted that Derek drove quickly even after being cautioned not to do so by the person who had the controlling influence on whether or not he would be allowed to drive motor vehicles. In addition, there were times when Derek found doing more than one thing at the same time to be difficult. For example, reading signs and driving were hard to do together. Of significance is the fact that Derek missed the last scheduled session with Mr. Brown because he forgot. Mr. Brown called Derek to leave a message about the missed session, but Derek did not return the telephone call. It is very significant that Derek missed the appointment with the one person who controlled his chance to drive motor vehicles and then after forgetting, he did not contact Mr. Brown right away when a message was left on his answering machine. When this development is taken into account along with Mr. Brown not seeing Derek drive while he was tired, angry, or depressed, one might have an understanding for the concern that family members had that Derek drives a motor vehicle. Therefore, it may be that in the future Derek may have more difficulty retaining the opportunity to drive motor vehicles. This is moreso the case when Mr. Brown stated that if he had been aware of Derek being very fidgety and gripping the door handle of the motor vehicle when Courtney Walker drove them into Toronto, he would have been more concerned about Derek driving. Derek had told Ms. Walker that he did not the f'ing city and that reggae music, which

they heard on the car radio, was nigger music. Historically, Derek did not express himself in such a racially inappropriate manner.

[31] Derek gambles at times. He plays cards and goes to casinos. The question might come to mind whether he should have the mobility opportunity of attending casinos or racetracks by being able to drive.

[32] Derek Gordon cannot go through a night with any assurance of not wetting the bed or soiling himself. He becomes frustrated with such events. It requires assistance from his mother most of the time to clean up the mess.

Constant Attendant Care for Derek Gordon

[33] Derek's senses have been diminished by the acquired brain injury so that he does not smell, sense temperature, taste food, or feel hunger. He has no appetite. He would not eat unless prompted to do so. Because he does not sense the taste of food or the sense of hunger, he cannot tell himself to stop eating because he does not feel full. Mr. Gordon must be careful with the temperature of the elements and of such products as hot water. He might scald himself in a shower. He might go outside in sub-zero temperatures wearing summer clothing again being unaware that he should be dressed in warmer clothing.

[34] Because of his limitations, he will require attendant care for the rest of his life. There is some disagreement about the quality of attendant care that should be allowed for Derek. He has some reluctance to accept assistance from caregivers. The defence suggests that a much lower level of damages be considered because of a lack of likelihood that the Plaintiff will accept the help. In effect, such funds might be wasted.

[35] I do not agree with the defence position regarding attendant care. Today, Derek receives attendant care assistance. The care is not a luxurious form of care. It is necessary. There is no likelihood that Derek will recover and have no need for this form of care. Acquired brain injury is permanent. It affects his frontal lobe. To suggest that the recommended attendant care be reduced significantly or abandoned completely simply passes the task over to the family in the sense of dumping such responsibility on them. There is no justification for doing so.

[36] Heather Muir is an occupational therapist who reported that Derek will require 24-hour a day attendant care for the rest of his life. With a person who is confused, disoriented, agitated, experiences physical aggression, poor memory, lack of insight and poor balance, and concrete thinking, one must anticipate that this Plaintiff will require a great deal of assistance. Such assistance would apply even when Mr. Gordon is out socially. His tendency to be inappropriate socially leaves him at risk of being engaged in physical confrontations. A blow to the head would be more hazardous for this Plaintiff than one who has not experienced a major brain injury.

[37] Ms. Muir pointed out that it is common for brain-injured clients to focus on the trial as the source of getting past all their accident-related problems. They think that the brain injury will go away after the settlement of the court case.

[38] Dr. Peggy Voorneveld, a neuropsychologist, described Mr. Gordon's functioning as that of a person with a lack of insight, not showing judgment, being irritable, not being able to make decisions and needing 24-hour a day care. She testified that a rehabilitation support worker is needed because such a person can diffuse a situation when such episodes as anger control and settling a person to a calm and controlled situation. Her work with Mr. Gordon revealed that all Mr. Gordon wants to do is get back to his normal life. The neurological findings discount malingering by this Plaintiff. The findings show frontal lobe injury resulting in impulsivity, inability to interpret situations, failing to complete daily hygiene requirements, and wearing the wrong clothing for the correct season. For example, Mr. Gordon might wear summer clothes in the winter.

[39] Dr. Hal Scher is an expert in clinical neuropsychology. He opined that Derek Gordon will require 24-hour a day attendant care for the remainder of his life. There is a significant risk for suicide. Also, there is danger of injury within the community by getting into fights in bars for example. Derek does not have self-control. The frontal lobes of human beings influence the regulation of emotions, disinhibition, and the ability to execute sequences of behaviour in order to complete events in a logical fashion so that goals can be achieved. Derek does not have the ability to control his frustrations. He can explode in anger as Dr. Scher pointed out at his office one day when Derek became very angry that the doctor met with his mother while Derek was out of the office having a smoke. Derek had gone out for a smoke when the doctor was ready to meet with him. He did not think Dr. Scher should have met with his mother. Within a short time, Derek became angry again when he was not able to complete a test that was being conducted by a psychometrist at Dr. Scher's office. Dr. Scher pointed out that Derek cannot control his emotions.

[40] Dr. Scher explained that suicidal ideation can be a symptom of severe depression and attention-seeking. Suicidal feelings are common with brain injuries. Derek will be frustrated and unhappy as he is isolated from his friends and unable to advance with a career as he experiences incontinence, impaired memory, and impulsive behaviour.

[41] Dr. van Reekum was qualified as an expert in the field of neuropsychiatric assessments. He testified that persons with acquired brain injuries are at greater risk of developing Parkinson's Disease and Alzheimer's Disease. He will be at risk of injury flowing from incidents of rage and problems with his balance. Mr. Gordon is at higher risk to experience depression that can become chronic. Dr. van Reekum thought that Mr. Gordon is at the greatest level of recovery that he will experience. Other factors that Mr. Gordon will experience will be a greater risk for weight gain, high blood pressure, stroke and heart attack. In his opinion, Dr. van Reekum thought that the Plaintiff needs on-going case management.

[42] Dr. van Reekum opined that attention deficit disorder is not a medical condition that never goes away. The Rule of Thirds applies to this disorder. One third of children with ADHD have symptoms into their adulthood, another one third have mild symptoms and function socially, and the final one third have no effects from the disorder. He did not agree with Dr. Bail's suggestion that the symptoms never go away. Dr. van Reekum thought that if Dr. Bail was reporting that Mr. Gordon was not at risk of harming himself, he must have been focusing on psychiatry but overlooking cognitive motor impairments. Those impairments are permanent. Mr. Gordon will have major depressive episodes during his life.

[43] I do not accept the evidence of Dr. Bail, a psychiatrist called by the defence. He suggested that Mr. Gordon might be a person with attention deficit hyperactivity disorder, that he could work, and that he was not impaired from doing daily activities. This appears to fly in the face of many other medical reports and opinions.

[44] In cross-examination, Dr. Bail accepted that he would defer to others such as neurologists and neuropsychologists. It appears to me that the others were more qualified than Dr. Bail to make opinions about the abilities of the Plaintiff to work and live alone. He ended up agreeing that he would defer to the opinions of Dr. Scher, Dr. Berry, Dr. Ouchterlony and Dr. van Reekum. They supported the opinions that this man could not live on his own independently.

[45] Dr. Bail appears to have attempted to cloud the proceedings with suggestions that some of Mr. Gordon's deficits could be attributed to attention deficit disorder and oppositional defiance disorder when there was not a foundation for such comments. He took these terms and opined that poor school marks and one suspension for smoking on school property is indicative of the possibility of the existence of these disorders. I might say that throughout Mr. Gordon's schooling there was no diagnosis of attention deficit disorder or oppositional defiance disorder. Dr. Bail had reported that ADHD never goes away, but in cross-examination he was faced with a medical journal report that as persons grow into their adult stage of life, 50% do not have any a continuation of the disorder and the other 50% have varying degrees of it. Dr. Bail then watered down his opinion by saying that he should have said that ADHD virtually never goes away.

[46] Dr. Bail suggested that Mr. Gordon might have been faking symptoms such as an altered gait, no taste, no sense of smell, and no sense of temperature. But he deferred to neurologists and neuropsychologists in making such findings. Dr. Bail had reported that Mr. Gordon had normal short-term memory meaning that he did not find any deficits, but then said that he did not mean that the Plaintiff did not have any memory impairment.

[47] The answers being given by Dr. Bail left the impression that he was gathering up terms, such as ADHD and oppositional defiance disorder, to create an impression that this Plaintiff had problems prior to the accident and that his inabilities should not be attributed to the accident. Dr. Bail went further to suggest that Mr. Gordon was malingering. When he was challenged on cross-examination, he did not have a foundation for such an opinion.

[48] Dr. Bail had a report from Dr. Sweeney, a neuropsychologist, describing Mr. Gordon's short-term memory deficits, and he did not agree with those opinions. Dr. Bail acknowledged that Dr. Sweeney had concerns that Mr. Gordon was a suicide risk, but then Dr. Bail obfuscated over whether Dr. Sweeney as a psychologist understood the matter. Dr. Bail had referred to the Plaintiff as a psychopath after a 1-hour interview. Dr. van Reekum did not have such an opinion. He was qualified to give expert opinion evidence in the field of neuropsychiatric assessments, but Dr. Bail attempted to suggest that Dr. van Reekum works in a geriatric facility and not with adolescents. These answers appear to me to be given by an expert who has become an advocate for the party calling him as a witness. That is not the role of an expert witness who is allowed to provide expert opinion evidence.

[49] Dr. Sweeney was referred to as a neuropsychologist retained by the defence to examine Derek Gordon. He was not called to testify at the trial. His report was relied upon in part by Dr. Bail for his opinions and report. The failure to call Dr. Sweeney to testify when his evidence would appear to have been at odds with the position taken by the defence may be taken into account whereby the trial judge can make an adverse inference against the Defendant in that the evidence of Dr. Sweeney would have adversely affected the case for the defence. I do infer a negative inference to the defence. I rely on the Supreme Court of Canada in *Levesque v. Comeau*, [1970] S.C.J. No. 55 at pg. 3.

[50] Dr. Donna Ouchterlony is an expert in the field of neurorehabilitation, who opined that Mr. Gordon is a high-risk person with a need for 24-hour a day supervision because he is hard to program to consistent actions. He is too variable. She thought that the Plaintiff would not respond well to any new circumstance in his life. I accept her testimony and that of the other expert medical witnesses for the Plaintiff that Derek Gordon requires constant attendant care.

Housekeeping and Home Maintenance

[51] Ms. Bierbrier costed in the need for assistance in housekeeping when Mr. Gordon lives alone. Currently, he resides at his parents' residence, but that will not last throughout his life. He will need on call service because of his limitations. This will include heavy cleaning and handyman assistance for home repair. The Cost of Future Care for Derek Gordon

[52] I am satisfied with the cost projections from the Plaintiff for future care, including attendant care and housekeeping and home maintenance, because they are reasonable and necessary. The categories of care set out by Carol Bierbrier are not a luxurious grouping of services for Derek Gordon. They meet the necessities of life for this young man, who cannot live out his life in a self-sustaining manner.

[53] On the other hand, the outlay completed for the defence by Lee Tasker appears to me to be focused on simply finding ways to reduce the dollars without consideration for the necessities that this man will require to live out his days. The defence for example suggests that because Mr. Gordon does not like attendant care providers around him, he will not use them in the future thereby making an allowance for them an excessive award of damages. Mr. Shulgan suggests

that allowance be made for 2 hours in the morning and 2 hours in the afternoon for attendant care. Further, if the Plaintiff needs attendant care when he sleeps, the use of an on-call service that might charge a minimum of 3 hours a night could be considered. Finally, any overnight attendant care can be provided by a moderately skilled caregiver at an hourly rate just over the minimum wage scale. To take the approach of Lee Tasker would be to gut the reasonable care for the Plaintiff and transfer the cost and responsibility to his family. I am not persuaded that he will not take the services recommended by Ms. Bierbrier; rather, I am satisfied that he will accept the help he needs. There may very well be times when he is upset with people trying to assist him, but he cannot go through life without help.

[54] The cost projections for the Bierbrier services calculated by Gary Principe are reasonable. The method of calculation is correct. The defence through Lee Tasker and Dr. Charette calculated future care costs at a much lower level than the Plaintiff did. I find that the defence approach to cost calculations is basically a slash and burn approach to reduce costs. The services recommended by Ms. Bierbrier are essential and do not amount to a wish list for a severely injured person to live out the remainder of his life.

[55] Ms. Bierbrier considered deficits in the cognitive area. Executive function, impulsivity, irritability, difficulty with following through with tasks, balance issues and fatigue are the main issues reviewed by Ms. Bierbrier. She has a balance of rehabilitative support workers and attendant care service providers so that duplication does not occur. Ms. Bierbrier was qualified as an expert witness for opinion evidence in the field of long-term care planning and costing of such services. She does not expect a support worker to do the work of an attendant care service provider. The rehabilitative support worker's responsibility is primarily to develop more community integration for the injured person. The individual oversees the activities of daily living. Often, people confuse the role of a rehabilitative support worker and a personal support worker. The latter has less training. The rehabilitative support worker works with the treatment team for the patient.

[56] Ms. Bierbrier does allow for on-call support workers for Derek Gordon so that he can call for assistance thus making allowance for some of Mr. Gordon's impatience with a person being by his side all the time.

[57] The bottom line is that Derek Gordon needs a person with him or available to him all the time.

[58] Ms. Bierbrier's costing of services sets out medication and assistive devices and equipment, professional medical services, rehabilitation services, attendant care services and housekeeping and home maintenance services. These were described in Exhibit 69. Mr. Principe's costing was set out in Exhibit 61E.

[59] The total for future care including attendant care costs as presented by Carol Bierbrier and Gary Principe are accepted in the sum of \$ 8,646,900. This takes into account the discount rates pursuant to the Rules of Civil Procedure.

[60] Future care costs are based on a reasonable chance that the losses will occur. See *Schrump et al. v. Koot et al.* (1977), 18 O.R. (2d) 337 (C.A.).

Future Income Loss for Derek Gordon

[61] Derek Gordon will not be gainfully employed during the remainder of his life. What might he have undertaken as a career? His school marks demonstrated that he did not perform very well in school. However, shortly prior to the accident, Derek advised his parents that he intended to complete his high school credits and take on an apprenticeship program for a tool and die maker. There had been an interest in doing mechanical work in heating and air conditioning as well.

[62] There is considerable debate between the sides as to whether or not Derek would have been employed in more than unskilled labour work. Witnesses regarding vocational prospects as well as experts in determining the value of a claim for loss of future income testified for both sides. Derek had worked for a company on a probationary basis in the tool and die making field. His brother, Trevor Gordon, is a contract tool and die maker. His brother-in-law, Garnet Leduc, is also a tool and die maker. His father and mother are university graduates. Even though he did not complete high school during the years of his teens, he did take community-based educational training working off credits for his high school diploma to the extent that he was 1 credit short. He would have had the opportunity to take on the tool and die making apprenticeship as a mature student. The defence position is that such a vocational career is wishful thinking on Derek's part; however, I am persuaded that there is a strong likelihood that he would have undertaken tool and die making as a working career. Why? He expressed an intension to do so before the accident. Secondly, he had worked for a tool and die making firm. His brother worked in the trade. The reason Derek appears not to have stayed with the company he was working on a probationary basis was that his girlfriend's father was the owner and he felt pressure being there. Joel Kumove pointed out that the influence on children by parents, who have gone on with post secondary education, is recognized as a positive influence in their offspring to go on to a career involving more than a high school education. The odds are in favour of Derek having done so. Further, he had done something about continuing his education and had openly told his parents of his intension. This is not a wish and a prayer career claim.

[63] George Boghean provided vocational assessment opinion evidence for the defence. He did not interview Derek Gordon. He formed his opinions from the records of Mr. Gordon. Mr. Boghean opined that Derek would not have become qualified for a trade such as tool and die making or a mechanic in the heating and ventilation trade. The Plaintiff would have worked in unskilled work such as a gas station attendant.

[64] Of importance is Mr. Boghean's subsequent opinion that if he had been provided with the information that Derek had worked after school and had continued his education through the Adult Learning Centre achieving good marks, he would have thought that there was a reasonable possibility of Derek entering a trade for a career. This effort at continuing his education demonstrated that Derek was motivated towards either tool and die making or heating and

ventilation trades. When Mr. Boghean had additional information that Derek was a hard worker as reported by a former employer, Mr. Kernaghan, and that he worked well with customers, he accepted that this was inconsistent with the information he had been given. Finally, with the reports from the parents about Derek stating that he was going to continue with his education and get a trade, Mr. Boghean agreed that there was a reasonable possibility of the Plaintiff taking on a trade and being employed for the rest of his life.

[65] Although Derek had not been a strong student, I am satisfied that he had made positive moves to gain an education and training to advance to a skilled trade such as tool and die making.

[66] The defence approach through Dr. Charette tagged Mr. Gordon as one who would do unskilled labour work, which I find to be unacceptable for the reasons already given. Further, the way that Dr. Charette calculated the income is not acceptable because he used a method that simply cut down costs without justification. For example, he used a median income for persons who had not finished high school working in unskilled labour careers rather than an average. This had the result of using a lower income figure by removing persons who are out of the active workforce for part of any given year. In addition, use of median incomes takes others out of the income calculation group. On the other hand, Mr. Principe used the average income of persons stating that it takes all persons within age groupings into account. Mr. Principe included persons employed in the skilled trades of tool and die making and heating and ventilation mechanics work. I am convinced that the approach of this Plaintiff as a skilled tradesperson is correct. Further, using the average incomes of persons in these fields of work as shown in Government of Canada statistics is reasonable and does not distort or inflate the income of Derek Gordon.

[67] Future income loss is based on a reasonable chance that the losses will occur. See *Schrump et al. v. Koot et al.* (1977), 18 O.R. (2d) 337 (C.A.).

[68] The value of the loss of future income is as calculated by Gary Principe in the sum of \$1,871,600 and is shown in Exhibit 61C.

Past Income Loss for Derek Gordon

[69] Mr. Principe calculated past loss of income for two scenarios, i.e. in the tool and die making and in the heating, ventilation and air conditioning fields. The former worked out to be valued at \$15,100 and the latter to be \$8100. The defence position was a past loss of income should be \$8,000 as an unskilled labourer without a high school education. As stated above, I find that it is most likely that Mr. Gordon would have become a tool and die maker. He had continued advancing his education through an Adult Training Centre. He told his parents that he was going to get a trade. The trade he had worked at was tool and die making. I do not make a negative inference to his leaving the firm in which he began working on tool and die making because he would appear to have left because the firm was run by his girlfriend's father and he felt pressured being there in those circumstances. This man was on course to do more than work as an unskilled non-high school graduate.

[70] The loss of past income is as determined by Gary Principe. This Plaintiff had a history of being employed and working regularly. The quantum is \$15,100. This is a net figure.

Discount Rates for Derek Gordon

[71] The discount rates used by Mr. Principe for both future care costs and loss of future income are 1% for the first 15 years and 2.5% for the remainder of the years. These figures have been calculated into the figures by Mr. Principe. There is no suggestion that there should be any variation in the discount rates for this Plaintiff. The sums of money awarded later in this judgment have made the discounts set out here.

Management Fee for Derek Gordon

[72] With the lack of cognitive skills and the amount of money involved for the damage award for Derek Gordon, there is no doubt that someone will have to manage his resources. The funds involved in this decision are multiple millions of dollars. He does not have the skill to manage his own financial affairs. He is in his mid-twenties. Mr. Gordon has a long life ahead of him. He needs all the help he can get to manage his money so that he has sufficient funds to live out his days. The fee is based on the future costs of care and future income loss awards determined in this trial.

[73] I have reviewed several cases with respect to management fees and considerations for what percentage of the award might be used. See *Mandzuk v. Insurance Corp. of British Columbia*, [1988] S.C.J. No. 100 (S.C.C.), *Arnold v. Teno (Next Friend of)*, [1978] S.C.J. No. 8 (S.C.C.), *Townsend v. Kroppmans*, [2003] S.C.J. No. 73 (S.C.C.), *Barosek (Litigation Guardian of) v. Turret Realties Inc.*, [2004] O.J. No. 1088 (C.A.), *Ligate v. Abick*, [1991] O.J. No. 1138 (Gen. Div.) and *affd at* [1996] O.J. No. 1244 (C.A.), *Coulombe (Litigation Guardian of) v. Beard*, [1995] O.J. No. 3162 (Gen. Div.), and *Kulack v. Babcock Estate*, [1994] O.J. No. 16 (Gen. Div.).

[74] A reasonable management fee is 5%. That will work out to \$525,925.

The Quantum of General Damages

[75] The cap now amounts to \$310,000. This represents the maximum that should be allowed for general damages. Often, the clearest example of a person who might qualify for the maximum would be a person who is left a quadriplegic person confined to bed for the remainder of the person's life. The defence submits that Mr. Gordon is not in that extreme a category to qualify for general damages at a maximum level. Mr. Shulgan submits that \$250,000 to \$275,000 would address this category of damages.

[76] I have noted that the extent of the injuries sustained by the Plaintiff is considerable. This man will not be employed again. He will have difficulty having an interpersonal relationship with a partner. His lack of control of bodily functions will be a constant source of frustration and mental distress. Mr. Gordon might not be confined to a wheelchair or a bed, but his acquired

brain injury has made him a captive of limitations to act as ordinary persons might. In effect, Derek Gordon is chained to a limited life in as major a way as one confined to bed. He has some mental processes, but they enable him to realize that he cannot function ordinarily and he cannot do anything about such shortfalls. Such a fate places Derek Gordon in the top end of the general damage award considerations because of these limitations. The general damages will be set at \$310,000.

Contingencies

[77] The defence submits that there should be a 20% contingency on the attendant care claim on the basis that Derek Gordon is not likely to accept this type of assistance. I am not convinced that there should be such a contingency when I take into account the evidence of many professional witnesses who stated that Mr. Gordon must have such attendant care. He might not like to have someone hovering about to help him, but the bottom line is that he does and always will need help. His brain injuries have taken away his ability to make his way through life on his own.

[78] With respect to consideration of contingencies regarding future income loss, I have accepted Mr. Principe's approach. He has taken into account contingencies in calculating average incomes, in allowing for unemployment and part-time employment. Basically, I find that there is no general or specific contingency to be taken into account.

Total Damages for Derek Gordon

[79] The total damages awarded to Derek Gordon are \$11,369,525 with the discount rates taken into account and broken down as follows less a deductible of \$15,000:

(i)	cost of future care and attendant care	\$8,646,900;
(ii)	cost of past wages lost	\$ 15,100;
(iii)	cost of future wages lost	\$1,871,600;
(iv)	general damages	\$ 310,000;
(v)	management fee at 5% on the above	\$ 525,925
(vi)	Total	\$11,369,525.
(vii)	Pre-judgment and post-judgment interest shall apply as per the Courts of Justice Act.	

Family Law Claims

[80] There are family law claims for loss of companionship and guidance by Dianne Gordon and William Gordon who are Derek Gordon's parents. The transcript will show the father as David and the pleadings as William. He is one and the same person.

[81] They have undertaken a tremendous role as caregivers. Dianne Gordon has been more involved in the day to day living of Derek. She left her job in order to be available to assist. At the same, William Gordon holds his job but assists daily.

[82] It is very remarkable how much these two people do for a disabled child of their relationship. It is a wonder that they have been able to maintain their own balanced mental health. They have experienced tremendous stress as they have assisted their son.

[83] The father explained how having caregivers in the home to attend to Derek is an inconvenience in the sense that they always have non-family members there. The parents' intimate relationship has become non-existent. He wonders about his stamina as he is always concerned about his son's safety, Derek's quality of life and how Derek will be taken care of when the parents are not able to assist.

[84] Dianne Gordon is the mother of Derek. She testified how she and her husband never get a complete night's sleep. She is very involved with the treatment. Mrs. Gordon has undertaken counseling since this accident. She uses sleeping pills and anti-depression medication. Since she is at home, she sees Derek become angry and punch holes in the closet door. Her son is not able to have a normal and constant relationship with female companions. It is disheartening to see her son not remembering his childhood.

[85] As I watched the parents while they testified, I would not have been surprised if one or both had lost their self-control. They have carried a tremendous burden with remarkable strength and are to be commended for helping their son.

[86] I am convinced that the damages for a family law claim for Dianne Gordon should be set at \$75,000 and for William David Gordon at \$75,000 net of a deductible of \$7500 each.

Contributory Negligence

[87] Derek Gordon was not wearing his seat belt. I have been advised by counsel that the parties have settled the issue of any contributory negligence that might be assigned to the Plaintiff. Whatever is the percentage agreed upon for Mr. Gordon's contributory negligence, the appropriate sum of money will be deducted from the total awarded to him.

Costs

[88] The parties may arrange for a further hearing with respect to costs if they are not able to agree upon them.

[89] If there should be an error in the figures referred to in these reasons or an item missed, the parties may raise that issue for further determination.

[90] The awards are jointly and severally made against both Defendants.

The Morrison Action

[91] Ryan Morrison is 25 years of age now. His injuries are catastrophic. He is a paraplegic person following major injuries to his spinal column at the T5-T6 level. He had no feeling below that level following the accident.

[92] Ryan was ejected from the motor vehicle. His consumption of alcohol on the night of September 26, 2006 was very moderate. He acknowledged consuming 1 bottle of beer in the vehicle while traveling to the accident site. He was to be the designated driver when the men were going to use his motor vehicle to go to the cottage of Cory Greig's parents. The change occurred only because his vehicle developed mechanical problems that necessitated a change of vehicles. He had volunteered to drive the Greig vehicle, but Cory Greig advised that he was fine to drive. Ryan did not notice any conduct of Mr. Greig to be unusual or to cause him to think that Mr. Greig was under the influence of alcohol.

[93] Ryan has developed his upper body strength with an aggressive exercise and rehabilitation program. There is a natural development of arm and shoulder strength because of the use of a wheelchair for mobility purposes.

[94] With the paralysis sustained by Ryan, he underwent surgery whereby repair work for his spine was undertaken. Rods were inserted to brace his spine. He has loss of bladder and bowel control. Dr. Phan from Sunnybrook Hospital found a complete T5 injury. Rehabilitation was undertaken at Lyndhurst. In March 2004 Ryan came home to his parents' residence. Changes to the physical layout of the residence of Karen and Peter Morrison were undertaken to make the home more user friendly for Ryan.

[95] In time, Ryan became aware of stem cell surgery that might assist him in being able to walk again or at least to be more mobile. Dr. Lima of Portugal was consulted about such surgery. After reviewing the medical reports, Dr. Lima was prepared to have the stem cell surgery performed on Ryan. Arrangements were made for Ryan and the family to go to Portugal for this surgery. The cost of travel, staying in Portugal and having the surgery completed was close to \$75,000. Fundraising was undertaken to have enough money for Ryan and the family to attend in Portugal for this medical procedure.

[96] When Ryan arrived in Portugal, Dr. Lima emphasized that there was no guarantee that the surgery would be successful to enable Ryan to walk or to have more mobility. Ryan was directed to other patients who had undergone the surgery so that he could consider further whether he wanted to go ahead with the process.

[97] The stem cell surgery was performed in late October 2004. Ryan returned to Canada and made arrangements for an aggressive rehabilitation program at the Rehabilitation Institute of Michigan in Detroit. Dr. Lima had recommended that Ryan work with the Institute in Michigan. A very active and hardworking exercise program was recommended.

[98] Ryan advanced from having no ability to control his lower body to being able to pull himself up to a hands and knees position, to being able to lift one leg up from the other when lying on his side, and to being able to crawl. He stayed at the Rehabilitation Institute of Michigan for two months in the spring of 2005 and returned 6 months later for further assessment.

[99] The Rehabilitation Institute of Michigan program is far more physical and demanding on the patient than standard physiotherapy. One has the impression that the expression of enduring pain for long term gain applies to this form of rehabilitation.

[100] Ryan works 5 days a week in this rehabilitation program. He uses leg braces to stand and move with parallel bars, a mechanized bicycle to develop the muscles of his legs and circulation, and a specialized treadmill for him to walk with the help of 3 other people after he is put into a harness to assist in raising him from sitting in his wheelchair to standing and walking. He works out in the swimming pool during the summer months by being lifted from his wheelchair in a harness into the pool.

[101] Ryan's treatment and daily routine were shown on Exhibits 91 and 94A and 94B. It is exhausting to read the charts and to listen to Ryan and others describe what he does. Exhibit 86 is a video of Ryan working at these exercises. The constant work of Karen Morrison, his mother, is remarkable.

[102] Most significant is the positive attitude that people working with Ryan report. It is evident in listening to Ryan testify that he is a dedicated and committed injured person who is not going to allow the injuries to overwhelm him. His mother provides tremendous support for Ryan in his daily program.

[103] Ryan does not have bladder and bowel control so that he must have assistance for a bowel routine and for bladder maintenance. He is not able to jump out of bed and go to the toilet at will. He uses a condom catheter to contain the flow of urine from the bladder, but it is not uncommon for the device to work loose and spill the contents of urine onto the bed during the night. He uses a commode wheelchair to shower.

[104] Ryan has qualified to drive a motor vehicle with hand controls. He has special devices to raise him to the seat level so that he can shift from his wheelchair to the lift and onto the vehicle seat. There is a robotic arm to move the wheelchair to the back of his vehicle.

[105] Ryan attends the gymnasium that his personal trainer runs, and he tends to the telephones as well as assists in the management of the facility. In reality, it is not employment. It is an activity that keeps him busy. In addition, Mr. Morrison takes the son of the personal trainer to the son's physiotherapy that he needs following a snowmobile accident.

[106] It is highly unlikely that Ryan will be able to work at an income-generating career at all. Because of his limitations, there would be heavy demands on prospective employers to adjust workplace environments to accommodate a person with the physical limitations Mr. Morrison experiences. The need for bowel treatments three days a week make it impossible to attend a work site for the commencement of a regular working day. His natural abilities are focused on hands-on work, but his physical limitations make it impossible to be engaged in such work. He did not like academic schoolwork. Any work he might seek would require him to be using his mental faculties in ways that have not been acceptable to him. Sales work might be a career that he could undertake naturally, but his physical restrictions make it virtually unlikely that he will be so employed.

[107] The reasons will be set out in the following categories:

- (i) Special damages;
- (ii) the loss of past income;
- (iii) loss of future income;
- (iv) the cost of attendant care;
- (v) the cost of future care;
- (vi) housing costs;
- (vii) whether there ought to be any adjustment in the discount rate to be applied with respect to each Plaintiff;
- (viii) any allowance for a management fee;
- (ix) the quantum of general damages;
- (x) any allowance for contingencies;
- (xi) the quantum of family law claims.

Special Damages for Stem Cell Surgery in Portugal

[108] Mr. Morrison undertook stem cell surgery in Portugal so that he might become more able to walk again. Dr. Lima testified that quadriplegic persons have a constant desire to walk again. This type of medical procedure was considered to be experimental. It was of interest to Mr. Morrison especially as he wanted to try any procedure that might assist him to walk again. Dr. Lima noted that Mr. Morrison was a complete quadriplegic patient. There was no guarantee of improvement from the surgery. Before going to Portugal, Mr. Morrison could not crawl. After

the surgery, he returned to Canada and arranged to attend the Rehabilitation Institute of Michigan in Detroit for an aggressive rehabilitation program as recommended by Dr. Lima. Dr. Hinderer from the Rehabilitation Institute of Michigan noted that Ryan Morrison progressed to the stage of being categorized as an incomplete paraplegic person who could crawl. Dr. Hinderer noted that all rehabilitation has some degree of pain experienced as the exercise is undertaken. It appears that there will have to be pain experienced if the patient is going to experience some benefit. The family went to Portugal with Ryan for this surgery. It was an example of family support for an injured member.

[109] Exhibit 86 is a video of Mr. Morrison in various stages of his exercise program including showing him moving himself up to a position of being on his hands and knees, something he could not do before going to Portugal for the stem cell surgery. Advancing to being able to crawl has been a major advancement. I am satisfied that this flows from the stem cell surgery.

[110] Undertaking stem cell surgery was a reasonable medical procedure that was not available in Ontario at the time Ryan Morrison undertook it. Such procedures were more in their early stages of development, but the operation and subsequent rehabilitation work has proven to be worthwhile in advancing Mr. Morrison's mobility. This procedure was reasonably foreseeable as a medical procedure with a substantial possibility of being undertaken and having a success likelihood.

[111] The cost of the procedure in Portugal is \$74,713.78 and is awarded as special damages.

Past Income Loss for Ryan Morrison

[112] I accept the figures for past wages for Mr. Morrison. He had a history of being employed as a willing worker. The figures presented will be the sum to be awarded. They are \$55,400.

The Loss of Future Income

[113] This man was not a strong student at any time during his educational years. The assessment of his academic skills has been that he simply did not like formal education. He was a hands-on person. Landscaping was a field of endeavour that could have become his career. On the other hand, the potential to work in his father's business and eventually to become the owner was considerable. He had attempted community college for the landscaping program at Niagara College, but he failed out of the program. There may have been a feature of homesickness at play in a first year in post-secondary education, but there was no doubt a continuation of not enjoying academic work. Although Mr. Morrison completed high school, his skills were tested out and found not to be at the level of a high school graduate. In other words, he was not functioning at the level of a graduate.

[114] Peter Morrison is the father of Ryan. He testified that he too was not a strong student when he went to school. Ryan had worked some of the time with his father. Peter Morrison owned the glass and mirror business with another partner. There never was any formality to Ryan buying out the business. Mr. Kumove is an expert in vocational assessment and rehabilitation. He expressed the opinion that there was a considerable likelihood that Ryan would buy out the business. Ryan was a person who liked to work with his hands. Oftentimes, the role of parents has a positive influence on career choices of children so that Dr. Doxey allowed for such an influence to take Ryan in the direction of the family business.

[115] Mr. Boghean for the defence had met with Ryan Morrison and thought that he would be able to work at such careers as a tele-receptionist, sales clerk, work in the security field, make leather products, and grind lens for eye ware. Dr. Doxey had the opinion that many of these careers were of the type that would be boring to Mr. Morrison. He was interested in school work nor did he like academic focused work. Dr. Doxey thought that it would be unlikely that Mr. Morrison could become employed.

[116] I accept that Ryan Morrison had a greater likelihood to enter and eventually purchase the business of his father and the father's partner. If he did, his income would have been projected to be \$44,000 for the first 3 years, \$64,000 for years 4-9 and \$84,000 for the balance of his working life. These are projections prepared by Ronald Smith. The father's income had not been accurate for income taxes, but Mr. Smith did a full review and determined that an income of this amount was reasonable. I accept Mr. Smith's calculations. Future income loss would work out to \$2,789,200 initially before any contingency and allowance for post-accident earning capacity.

[117] I conclude that Ryan Morrison will not likely become employed during the balance of his life because of his physical limitations and his natural lack of skills with academic types of employment.

[118] Mr. Vigmond recognizes that Ryan Morrison has worked at the gym of his personal trainer answering the phone and doing very limited tasks. He recommends that there be an allowance of \$72,500 for post-accident earning capacity based on \$8 per hour for two and a half hours a day for two or three days a week. Based on a 50-week year and a present value factor from Mr. Smith of 29, the allowance of \$72,500 is determined. I accept this submission noting that I doubt that Mr. Morrison will be employed in the future.

[119] Mr. Vigmond accepts a 15% contingency be deducted from future income loss for potential work that he might be able to do taking into account the evidence of Mr. Boghean and Dr. Doxey as well as Joel Kumove. A 15% contingency allowance on \$2,789,200 works out to \$418,380.

[120] When the allowances are made for post-accident earning capacity of \$72,500 and a 15% contingency on future income loss of \$418,380, the net is \$2,298,320 for future income loss.

[121] When future income loss of \$2,298,320 is added to past income loss of \$55,400, the total for income losses will be \$2,353,720.

[122] As in the Gordon reasons, I rely on *Schrump et al. v. Koot et al.* (1977), 18 O.R. (2d) 337 (C.A.) for the reasoning that an award for future income loss is made out if there is a reasonable chance that the losses will occur.

Attendant Care

[123] Ryan Morrison will require constant care throughout his life. There is no way that he will be able to live on his own without assistance. Today, he lives at his parents' residence, but he would like to have his own residence someday. There will come a time when his mother and father will not be able to provide the help they do today.

[124] In his parents' residence, there are limitations with the physical plant. For example, he does not have so large a bedroom that he can maneuver around the bed with the wheelchair. He cannot reach the supplies he requires. Mr. Baum is an expert in residential facilities for persons with the limitations that Mr. Morrison has.

[125] Even with the home designs described by Mr. Baum, Ryan Morrison will need assistance. I am not persuaded by Lee Tasker for the defence that he does not need the attendant care recommended by Carol Kelly. Lee Tasker recommended a very streamlined care with unskilled persons. I am convinced that this Plaintiff will need help from people who are more than unskilled minimum wage persons. I appreciate that other cases have concluded that unskilled persons can do the job of watching out for persons such as Mr. Morrison during the night. The defence has suggested that a service could be on call arriving within about a half an hour of a call for assistance. Ms. McCarthy from Service Comfort Keepers testified that he company can supply personal support workers who can attend on call, who can be present in the residence either as an awake person throughout the night or as a person who can go to sleep during the night at the residence of the patient. The time to come to the residence to assist Mr. Morrison would be a half hour following a call to a life line centre. The life line centre then would have to call this company. The cost is \$23 per hour with a minimum charge of 3 hours time per call. Ms. McCarthy did not have information about whether or not the personal support workers from her company had any skills to work with spinal cord injury persons. This approach by the defence with the opinions of Lee Tasker appears to be shortsighted. If this Plaintiff were to be living on his own and did not have 24-hour a day attendant care, he would be at risk of not having an emergency addressed quickly. For example, if there were a fire and it was going to take a personal care worker half an hour to come to his residence, the Plaintiff might die in the meantime. This Plaintiff is not one who only needs a nanny to pick up after him. He needs someone who can be there right away and someone who understands the limitations of a spinal cord injury person so that he can be assisted properly.

[126] In a nutshell, Ryan Morrison does not fall within the considerations given by Zuber J. in *Dube (Litigation Guardian of) v. Penlon Ltd.* (1994), 21 C.C.L.T. (2d) 268 or by

Spiegel J. in *Desbiens v. Mordini* [2004] O.J. No. 4735. In those cases, the trial judges determined that unskilled persons with minimum wages could provide overnight security. I am not persuaded that Ryan Morrison falls into the same situation as the plaintiffs in both those cases. The facts in Mr. Morrison's case call for more than a person without skills.

[127] The costs for such care as determined by Carol Kelly and Ronald Smith are reasonable. They are challenged by the defence as excessive. Lee Tasker, for the defence, has provided her evidence to reduce those dollars, but her assessment amounts to searching for any way to knock down the dollars without proper foundation. Her suggestions would amount to downloading the work to the family of Ryan Morrison. That is not fair to those family members.

[128] Mr. Kaul was called by the defence as an occupational therapist to provide evidence that the attendant care suggested by the Plaintiff is greater than is needed. He appears to have been presented because he had seen Ryan Morrison whereas Lee Tasker had not interviewed the Plaintiff. It is of importance that the notes he made from his meetings with Ryan Morrison were destroyed purposefully by Mr. Kaul. He justified doing so as a matter of maintaining the privacy concerns of the Plaintiff. He testified that it is his practice to only keep his notes until he completes his final report. I find this practice to be almost unbelievable today when professional persons take great care to maintain records. The destruction of the notes reduces the value of the evidence of Mr. Kaul so that I place very little weight in it.

[129] The cost of 24-hour a day attendant care as presented by the Plaintiff is accepted. The total awarded is included with the cost of future care below.

The Cost of Future Care for Ryan Morrison

[130] Carol Kelly is a highly qualified occupational therapist who presented a future care plan for Mr. Morrison. I find that the categories of care are reasonable in the quantity of services and the value of such services. I do not accept the suggestion that they are excessive. For example, the defence suggested that the cost of a residence for Mr. Morrison could be different by having a one level residence rather than having two levels with an elevator to enable Mr. Morrison to move from one level to the other. Mr. Baum pointed out that to have a one level residence would require more land so that the cost of such a residence would be in excess of \$600,000. This is an attempt to suggest a reduction but without foundation.

[131] Ms. Kelly is a certified Life Care Planner with special skills to develop the plan for Mr. Morrison's future. Her skills are greater than those of Ms. Tasker whose primary task appeared to be to whittle down the dollar figures for the future care of the Plaintiff. The Kelly evidence addresses 8 headings for the costs of future care with Ms. Kelly handling 7 and Jeff Baum the 8th for housing. The 8 headings for future care are:

- (i) activities of daily living;
- (ii) mobility considerations for the wheelchair, repairs, a walker and leg braces;
- (iii) transportation with the Plaintiff's own vehicle with modifications;
- (iv) rehabilitation with various treatments such as physiotherapy, chiropractor and massage therapy;
- (v) health maintenance and supplies;
- (vi) medications and supplies;
- (vii) housing recommendations as given by Jeff Baum;
- (viii) future considerations such as a power wheelchair to avoid bursitis and wear of the shoulders, the possible need for an overhead lift if Mr. Morrison is unable to slide on a board from his bed to a wheelchair, and the need for attendant care if he is ill or bedridden.

[132] Ms. Kelly separated Mr. Morrison's life into 3 time periods. First, there is the period from the time of the trial to May 2008 when she expects he will move from the home of his parents. Secondly, there is the period from May 2008 to May 2023 when the Plaintiff will be 41 years old and 20 years after sustaining the injuries. Thirdly, the period of May 2023 to the end of his life. This last time frame will accommodate shoulder deterioration when full assistance for transfer, overhead and reaching tasks and lifting will be required. Ms. Kelly worked on the assumption that power equipment will be used by Mr. Morrison for mobility with independence being maintained in a wheelchair.

[133] These 3 periods of time take into account attendant care for the following:

- (i) dressing;
- (ii) undressing;
- (iii) putting on and removing shoes with clothing changes at times of incontinence;
- (iv) long leg braces being put on and taken off as well as being adjusted;
- (v) transfers;
- (vi) getting into and out of bed for naps;
- (vii) bowel treatments on and off the commode;
- (viii) incontinent episodes;
- (ix) supervision during bowel treatments;
- (x) supervision during showering;
- (xi) supervision for extra showering during summer months;
- (xii) overnight supervision during weekdays and weekends;
- (xiii) bathroom cleaning after use;
- (xiv) bedroom cleaning;
- (xv) meals, snacks, transporting hot food or liquids or heavy food items;
- (xvi) personal laundry;
- (xvii) cleaning Mr. Morrison after incontinence experiences, catheterization and retrieving supplies;
- (xviii) cleaning equipment with respect to bowel routine;
- (xix) work on the Giger MD (an aerobic exercise device);
- (xx) walking with the parallel bars;
- (xxi) electrodes applied and removed for the cycle;
- (xxii) Pneu-weight walker which requires 4 people to use properly;

(xxiii) Checking skin care for pressure sores and application of creams;

(xxiv) Ordering and monitoring supplies;

(xxv) Ensuring that equipment is kept in good working order and cleaned.

[134] Ms. Kelly did not agree with Ms. Tasker's suggestion that spinal cord injury people are similar to AIDS persons. Ms. Kelly stated that AIDS persons are palliative with nursing and with full protection from bodily fluids, significant health issues for the skin, vision and sores in the mouth. Often, AIDS patients' families provide the care because no one else will do it. Further, Ms. Kelly thought that Ms. Tasker was wrong to value the costs for attendant care in a similar category to a nanny because there is no similarity. This was an area in which Ms. Tasker projected fewer hours for personal support worker assistance.

[135] Ms. Kelly projected that the wheelchair be replaced every 5 years. This was founded on standards from the Ministry of Health.

[136] Ms. Kelly thought that Mr. Morrison would change his motor vehicle every 4-5 years based on the recommendations of the auto dealership. The average for people is 5-7 years.

[137] On the other hand, Lee Tasker thought that a vehicle would last Mr. Morrison 7-11 years with proper maintenance. She had spoken with the dealership that supplied the current vehicle to the Plaintiff.

[138] Ms. Tasker thought that Mr. Morrison should be able to use a wheelchair longer than suggested by Ms. Kelly.

[139] Ms. Tasker projected that Mr. Morrison would be likely to move twice in his lifetime, and Ms. Kelly projected 4 times. This was based the thinking that the Plaintiff would move once from his parents' residence and then one more time during his lifetime based on the assessment that a person with the physical disabilities of Ryan Morrison will be likely to move less than fully mobile people because it is more difficult for persons with physical handicaps to move. Ms. Kelly used the estimate of 4 moves because that is the average for Canadians to move during their lives. At the closing submissions, Mr. Vigmond recommended 3 moves.

[140] Lee Tasker used a family therapy model when developing her report on attendant care and future care costs. This model was not adapted to a personal injury plaintiff by its creators. She explained that she thought it could be applied to Mr. Gordon and Mr. Morrison. There does not appear to be a foundation for making such a conclusion; rather, she seems to have latched onto this model without giving consideration to her thinking that Mr. Morrison would marry and have children. The family model adopted by Ms. Tasker is broken into 9 life stages and 4 career stages in a person's life. They are the following:

(i) living independently (as in moving from his parents' residence);

- (ii) becoming independent;
- (iii) marriage;
- (iv) child bearing family stage;
- (v) family with pre-school children;
- (vi) family with school-aged children;
- (vii) family as the children move out;
- (viii) the empty nest stage;
- (ix) retirement to death.

[141] The career stages are the following:

- (i) exploration i.e. looking for career opportunities;
- (ii) the establishment phase—getting established and then skill building;
- (iii) the maintenance phase of the career;
- (iv) disengagement or retirement phase.

[142] The 13 phases adopted by Ms. Tasker appear to have been used for a unique setting in family situations. A paraplegic person, who has little likelihood of being employed and whose marriage would have considerable pressures upon it, would seem to me to be outside the norm. Ms. Tasker takes it as a given that there will be a marriage.

[143] I conclude that it is best to look at the man involved, i.e. Ryan Morrison, and not to give weight to this family model. Where is Mr. Morrison at this stage in his life and where might he reasonably go? He simply is not going to have a normal life.

[144] The defence went through the items for future care line by line questioning each. Suggestions were made that the dollar value for some items might be less or that less attendances by physiotherapists, personal trainer, the occupational therapist and others. The end result is that one might go through these items and whittle away at them, but there would only be a major reduction if one were to conduct a major reduction in services. As I have stated previously in these reasons, the outcome of such a harsh excising of assistance to Mr. Morrison would be to shift the cost and responsibility to members of his family. This is not a self-insuring regime. Rather, the assessment of damages is a process to provide reasonable and needed compensation

for injuries sustained along with treatment and care. In this case, the care is for the rest of Mr. Morrison's life.

[145] Dr. Charette, working with Lee Tasker, presents a marked reduction of dollars for the future care and attendant care costs. He recalculated the Carol Kelly figures in part by not using the discount figures used by Dr. Coyte. My impression is that he clouded the scene with general discussion about how Dr. Coyte was wrong about the costs of future care health as if that makes much of the Kelly approach unacceptable. It appears to me to be a lot of smoke and mirrors. The approach by Carol Kelly and Dr. Coyte has substance, is well-founded and makes sense. It is not an exorbitant calculation that ought to be discarded with a pass of the hand.

[146] There is some substance to the circumstance that Lee Tasker did not meet Ryan Morrison or his family. They lost the benefit of first-hand knowledge that Carol Kelly and Joel Kumove had. The lack of face-to-face meetings with Ryan Morrison and his family led to a weakening of any assessment of the Plaintiff. It then reduced the value of Ms. Tasker's report when Dr. Charette worked through his calculations. I was left with the impression that both Lee Tasker and Dr. Charette were left to struggle and in effect try to talk a good line with as much obfuscation as possible when challenging the Plaintiff's witnesses who had seen and met with Mr. Morrison.

[147] Dr. Empringham was called by the defence to provide evidence on mortality tables for a person in Mr. Morrison's position of being injured and also being a smoker. His expertise is with the life insurance industry in predicting life expectancies. Dr. Empringham was of the opinion that Mr. Morrison would have a shorter life expectancy because of his injuries and even more of shorter life expectancy because he smokes tobacco products.

[148] Ronald Smith had calculated out the costs of future care costs in the plans of care presented by both Carol Kelly and Lee Tasker. He did additional calculations for the mortality considerations presented by Dr. Empringham.

[149] The future care plan developed by Carol Kelly is accepted. That includes attendant care. The calculations conducted by Ronald Smith, with the allowance for mortality given by Dr. Empringham, are to be used. Inclusive of future considerations in the Carol Kelly plan, the total awarded for future care costs will be set at \$8,880,800.

Housing Costs

[150] This was a separate consideration from the Carol Kelly report and the calculations from Ronald Smith. Jeff Baum, from Adapt-Able Design Group, is a specialist in housing for persons with physical limitations such as Ryan Morrison. He prepared a report that addresses the needs of a person with paraplegia so that the individual might live in a barrier-free home setting. The concept of removing barriers focuses on enabling a human being to function much more self-reliantly. For example, today in his parents' residence, Mr. Morrison has limitations in moving around his bedroom and moving to the shower area.

[151] Mr. Baum took into account rental housing, purchasing an existing residence or purchasing a new residence. Rental housing appears to be a remote idea because many landlords would not want to make changes to their property that would be required for Mr. Morrison. In addition, rental housing is usually short term so that the Plaintiff could find himself having to move after undertaking many adjustments to get settled into the residence.

[152] An existing house carries with it baggage in the sense that modifications must be made to accommodate a person with the limitations Mr. Morrison has. For example, an elevator or a lift would be required. It may be difficult to find an existing residence that could be altered so that an elevator could be installed. If an elevator could not be put into the residence, at the very least a vertical platform lift would be needed so that Mr. Morrison could move from floor to floor as well as from the entrance. Mr. Baum's experience leads him to recommend a two or three-stop residential elevator.

[153] It appears that a new residence would be the type of accommodation that could meet Ryan Morrison's requirements as it is being built.

[154] Mr. Baum testified that the best location for such an elevator would be adjoining or partially located within the garage of the residence.

[155] A mudroom is a must for wheelchair access. This would necessitate making changes to an existing residence main floor laundry/ mudroom or making a new mudroom.

[156] An emergency exit would be needed should there be an emergency that would not let Mr. Morrison out the primary entrance.

[157] Mr. Baum recommended a two-story residence with an elevator to enable Mr. Morrison to move from floor to floor. The lower level would be the exercise equipment area. Ryan has a great deal of exercise equipment so that he needs space to house it. There is no suggestion that Ryan Morrison has too much exercise equipment. He engages in a greater exercise program than many people. In fact, he does more exercise than many who do not have limitations on their ability to move. It would be a mistake to take away the ability to exercise. It benefits him. He must be dedicated to the tasks in order to do them.

[158] Mr. Baum laid out the requirements for the balance of a residence that would have counters at a level conducive to use by Mr. Morrison. The shower facility would be designed to enable him to use the commode wheelchair for showering. A swimming pool would provide the opportunity to exercise during the summer months. He suggested that a hot tub might be considered as well. A lifting device to assist in transferring between the wheelchair and the water would be required.

[159] The cost of housing includes an allowance for a change of residence a number of times during Mr. Morrison's life. Ms. Kelly allowed for 4 moves including the first from his parents' residence. Mr. Vigmond suggested that an allowance be made for 3 moves. Ms. Tasker thought 2 moves should be budgeted because a person with the physical limitations of Mr.

Morrison would be likely to remain in a residence longer than others due to a desire to avoid change.

[160] Doorways in a residence will have to be wider to enable the use of a wheelchair

[161] Flooring in a residence will have to be wood, ceramic or vinyl material or low pile carpet so that a wheelchair can move easily.

[162] Air conditioning, an intercom system are recommended. An emergency power source such as a gas-powered generator is in the category of a necessity should there be an extended power failure for the purposes of operating an automatic door opener, a residential elevator and central air conditioning.

[163] Mr. Baum was of the opinion that total housing costs for an existing residence would cost in the range of \$406,800 to \$501,900.

[164] A new residence would cost in the range of \$427,900 to \$449,300.

[165] Whether a new or existing residence, Mr. Baum noted that his costs projections excluded land transfer tax, moving costs, property taxes, utilities, an annual maintenance contract for a lift service, a security system monitoring fee, home maintenance and a hot tub and swimming pool.

[166] Mr. Baum gave consideration to the mobility of Canadians. Most people move within a short distance from where they have been living. The younger the person, the more he or she is likely to move more often. As people age over 30 years, they tend to move less. Carol Kelly advised that on the average Canadians move four times during their lives. Lee Tasker suggested that there only be an allowance for two moves because a person with physical limitations would be more likely to move fewer times. Mr. Baum thought that Mr. Morrison would move one or two times after moving from his parents' residence.

[167] The extraordinary housing costs for Ryan Morrison are projected by Mr. Baum to be \$145,000 to \$165,000 for each future move.

[168] Mr. Vigmond submitted that there be an allowance for three moves, including from his parents' residence initially, and that he be awarded \$374,800.

[169] I am persuaded by the evidence of Mr. Baum, as an expert in this field, that the costs he projects are reasonable. I accept the suggestion that Mr. Morrison will likely move fewer times than Canadians without mobility limitations. It is reasonable that he will move in the not too distant future to his own residence and that it will likely be in the Bradford area where he has lived throughout his life. Mr. Morrison is only mid-twenties. I would anticipate that it is reasonable that he will move another two times. The sum of \$374,800 is a reasonable allowance and will be awarded as part of the judgment.

Discount Rates

[170] Rule 53.09 of the Rules of Civil Procedure provide for automatically calculated discount rates. For the purposes of this trial, the discount rate for the first 15 years works out to 1% and for the years thereafter the rate is 2.5%. However, Dr. Coyte provided expert opinion evidence in the field of health economics. He has extensive qualifications in this field covering several decades. He was able to point out that the discount rates set out in Rule 53.09 may have an inaccuracy for the costs of health care when determining what value to assign for such costs in future care.

[171] There was some dispute between Dr. Coyte and Dr. Charette on this topic. Dr. Charette thought that Dr. Coyte was including some features that were distracting the calculations of such costs. The end result for Dr. Coyte was that the discount rate should be 0% for the first 15 years and 1.5% for the years thereafter.

[172] Dr. Coyte pointed out that costs of future care would be on a conservative basis 30% greater than the rate of inflation in general. He testified that health care expenditures increase more than twice the rate of inflation in the last 40 years.

[173] Dr. Charette on the other hand stated that Dr. Coyte did not look at prices but rather at expenditures when considering health care costs for the future of Ryan Morrison. Expenditures are the combination of price multiplied by the quantity for example as expenditures for computers. Over the years, the expenditures for computers have increased while the price per computer has declined. Dr. Coyte was only looking at expenditures for total costs of health care in Canada. Dr. Charette criticized Dr. Coyte for talking mostly about historical trends in Canada rather than talking about prices. In his opinion, Dr. Charette did not think that the price of health care services would increase above the rate of inflation so that there would be no basis for varying the discount rates set out in the Rules of Civil Procedure.

[174] I do not agree with the critique of Dr. Charette about Dr. Coyte's opinions regarding discount rates and health care costs in the lifetime of Ryan Morrison. Dr. Coyte explained that in the health care field, one approaches expenditures on a broader scale than in other fields noting that Dr. Charette is an economist in fields other than health-related economics. In other fields, one might tend to simply multiply price times the quantity of product involved. However, in health related expenditures, Dr. Coyte pointed out that the examiner should look at such components as the unit price per quality, the quality of the product, and service intensity. Looking at future care needs requires the observer to focus the financial liability exposure and how that liability exposure is likely to occur in the future. How will the standard of care change in time? How will the price per unit quality change over the coming years? If there were to be an exclusive view on only the unit price, it will not address the financial liability exposure implications during the remaining years of Mr. Morrison's life. The aging of the population, population growth, the underlying rate of inflation and changes in service intensity all influence future health care costs. For example, during the past few decades the quality of services with significant innovations with x-ray machines, CT Scanning, MRI,

PET Scans to mention a few have expanded considerably. Dr. Coyte testified that when the quality of health care improves, there is an inflation of costs that accompanies them.

[175] When Dr. Coyte did his report, he noted that he had worked with an erroneous discount rate for the Rules of Civil Procedure. He corrected that. However, Dr. Charette found that it was hard to fathom how Dr. Coyte could have that error and still come out with his rate of health care inflation being 30% greater than the general rate of inflation. There were many words of comment from Dr. Charette on this point, but I do not think the criticism carries weight. Dr. Charette spoke as if Dr. Coyte was manipulating figures and percentages at will if he could do a calculation with the wrong discount rate. The bottom line is that no matter how anyone looks at the health care figures, Dr. Coyte constantly stated that health care costs in the future will be 30% greater than the general rate of inflation. All he had to correct on the topic of the discount rate was to apply the 30% to the correct discount rate in Rule 53.09. He had used 3% in the first 15 years rather than 1%. When he worked in his 30% considerations, the result was that the discount rate should be lowered in the first 15 years to 0% and 1.5% for the years following.

[176] Dr. Coyte is the expert in the field of health economics. I find that his expertise carries the day over the opinions of Dr. Charette. In the end, I am satisfied with the projection that health care costs in the future will rise at least 30% faster than the general rate of inflation.

[177] The discount rate for future health care costs will be discounted at 0% for the first 15 years and 1.5% for the years following. That adjustment in the discount rates has been incorporated into the costs for attendant care and future care set out above.

Management Fee for Ryan Morrison

[178] A management fee will be awarded in the sum of 4% for future care and future income loss because there is a need for a professional person to manage the large award in this action. Mr. Morrison has a physical limitation that will impede his ability to handle such a sum of money. In addition, the Plaintiff does not have the education or training to invest and management this sum of money. The Plaintiff has sought a 4% management fee, and I am satisfied that such a fee is warranted. The fee will be \$447,164.80.

[179] As in the Gordon reasons, I have reviewed several cases with respect to management fees and considerations for what percentage of the award might be used. See *Mandzuk v. Insurance Corp. of British Columbia*, [1988] S.C.J. No. 100 (S.C.C.), *Arnold v. Teno* (Next Friend of), [1978] S.C.J. No. 8 (S.C.C.), *Townsend v. Kroppmans*, [2003] S.C.J. No. 73 (S.C.C.), *Barosek (Litigation Guardian of) v. Turret Realities Inc.*, [2004] O.J. No. 1088 (C.A.), *Ligate v. Abick*, [1991] O.J. No. 1138 (Gen. Div.) and *affd* at [1996] O.J. No. 1244 (C.A.), *Coulombe (Litigation Guardian of) v. Beard*, [1995] O.J. No. 3162 (Gen. Div.), and *Kulack v. Babcock Estate*, [1994] O.J. No. 16 (Gen. Div.).

General Damages

[180] The cap for general damages for pain and suffering is \$310,000. As mentioned in the reasons for the Gordon action, the clearest example of a person who might be considered for the maximum general damages is a person limited to surviving as a quadriplegic person. Ryan Morrison is a paraplegic person, who strives to be active with a tremendously aggressive rehabilitation program. Mr. Shulgan suggests that the award should be less than the maximum and recommends that the amount be \$265,000-\$290,000. On the other hand, Mr. Vigmond recommends that the maximum be allowed. Although Mr. Morrison is not confined as a person experiencing quadriplegia would, he is dramatically limited. If he did not have the positive attitude that he has, he would no doubt be far more confined to a sedentary life. For most people, the less active and demanding physical routine would be the case. His condition leaves him susceptible to bowel and bladder problems for the rest of his life. Spasticity is a regular complication as he attempts to move. He is at risk of long-term kidney damage and deep vein thrombosis. Urinary tract infections are a constant possibility. Osteoporosis is a future possibility. I am satisfied that this damage heading should be set at the maximum in the sum of \$310,000.

Contingencies

[181] I do not find there is any general or specific contingency to be applied here for the cost of future care. The mortality rate evidence from Dr. Empringham has already been taken into account by Ronald Smith in calculating the damage figures. He based his determinations upon the effect of the injuries impacting on the length of life for Mr. Morrison as well as the fact that Mr. Morrison smokes tobacco products. His life will be shorter because of his injuries and also because he smokes tobacco products. I have already taken into account a 15% contingency on the loss of future income.

Total Damages for Ryan Morrison

[182] The total awarded to Ryan Morrison will be \$ 12,441,197.78 with the discount rates taken into account, with a deductible of \$15,000 and broken down as follows:

(i) special damages	\$ 74,713.78;
(ii) loss of past income	\$ 55,400
(iii) loss of future income	\$2,298,320
(iv) cost of attendant care and future care	\$8,880,000
(v) housing costs	\$ 374,800
(vi) management fee at 4%	\$ 447,164
(vii) general damages	\$ 310,000

Total

\$ 12,441,197.78

Family Law Claims

[183] Karen Morrison is the mother of Ryan. She has provided tremendous care and assistance to her son since the motor vehicle accident. The degree of loss of care and companionship as well as guidance that she might have had is astronomical. How she can continue with the daily activities as set out in Exhibits 94 A & B is almost hard to fathom. It is as challenging to understand as I find comprehending how Ryan keeps up the active rehabilitative program he does. He might very well never have continued without the support of his mother.

[184] There will be a family law award of \$75,000 net of a deductible of \$7500 to Karen Morrison.

[185] Peter Morrison is Ryan's father. He has worked extensively with his son, but to a lesser degree as he continues to work at his own business with his partner. He had a good relationship with his son prior to the accident. As set out earlier in these reasons, it is likely that Ryan would have worked with his father and taken over the business. The loss of companionship and guidance from his son has been and will be significant.

[186] There will be a family law award of \$60,000 net of a deductible of \$7500 to Peter Morrison.

[187] Rochelle Morrison is the sister of Ryan Morrison. When the action commenced, she was a minor but is now an adult. Her testimony indicated that she and Ryan had a healthy relationship as sister and brother. The severe injuries sustained by Ryan no doubt have had a major impact on Rochelle. She has not had the same relationship since the accident.

[188] There will be a family law award of \$ 30,000 net of a deductible to Rochelle Morrison.

Contributory Negligence

[189] Ryan Morrison did not have his seat belt fastened. I have been told that the parties have settled the issue of contributory negligence for Ryan so that the percentage of negligence will lead to a reduction in the sums awarded.

Costs

[190] The parties may arrange for a further hearing on costs if they are not able to work out costs between them.

[191] If there is any error in the figures or if any item has been missed in these reasons, the parties may raise that issue for further determination.

[192] The awards are jointly and severally made against both Defendants.

Justice B.A. Glass

Released: January 23, 2007

COURT FILE NO.: 04-B7133

DATE: 2007-01-23

ONTARIO

SUPERIOR COURT OF JUSTICE

B E T W E E N:

Derek Gordon, Dianne Gordon & William Gordon

Plaintiffs

- and -

Cory Greig & Ford Credit Canada Leasing
Company

Defendants

COURT FILE NO.: 04-B7066

Ryan Morrison, Karen Morrison, Peter Morrison &
Rochelle Morrison by her Litigation Guardian
Karen Morrison

Plaintiffs

- and-

Cory Greig & Ford Credit Canada Leasing
Company

Defendants

REASONS FOR JUDGMENT

Justice B.A. Glass